June 16, 2025

SUBMITTED ELECTRONICALLY VIA *http://www.regulations.gov*

Secretary Robert F. Kennedy, Jr.
U.S. Department of Health and Human Services
200 Independence Avenue, SW
Washington, DC 20201

Steven Posnack
Acting Assistant Secretary for Technology Policy
Acting National Coordinator for Health, U.S. Department of Health and Human Services
330 C Street, SW
Washington, DC 20201

Stephanie Carlton
Deputy Administrator of the Centers for Medicare & Medicaid Services
U.S. Department of Health and Human Services
200 Independence Avenue, SW
Washington, DC 20201

***Re:* [CMS-0042-NC] Request for Information; Health Technology Ecosystem**

Dear Administrator Oz:

Pfizer Inc. (Pfizer) appreciates the opportunity to submit comments on the Health Technology Ecosystem RFI. Pfizer is a global pharmaceutical company that is in relentless pursuit of breakthroughs that change patients' lives. We innovate every day to make the world a healthier place. Our purpose ensures that patients remain at the center of all we do. We live our purpose by sourcing the best science in the world; partnering with others in the healthcare system to improve access to our medicines; using digital technologies to enhance our drug discovery and development, as well as patient outcomes; and leading the conversation to advocate for pro-innovation/pro-patient policies. Every decision we make and every action we take is done with the patient in mind—and to nurture an environment where breakthroughs can thrive.

Question PC-5: What can CMS and its partners do to encourage patient and caregiver interest in these digital health products? This includes asking whether CMS should review or approve digital health products on the basis of their efficacy, quality, or impact on health outcomes and what policy or reimbursement changes would increase adoption.

Pfizer Comment: Digital health products, and in particular prescription digital therapeutics (PDTs), have been shown to improve clinical outcomes and reduce overall health spending.[1] CMS should recognize that current pricing levels, including the reported $129 reimbursement by some Medicare Administrative contractors (MACs)[2][3] for Digital Mental Health Treatments (DMHTs), are insufficient to support the development, adoption, and sustained use of high-quality digital therapeutics. To encourage uptake by patients and caregivers, CMS should:

- Set an adequate national price floor for DMHT codes, specifically G0552, to ensure adequate reimbursement across MAC jurisdictions.
- Establish product-specific or category-based pricing to account for the variation in development cost and clinical impact of digital therapeutics (e.g., treating anxiety versus insomnia).

Question PR-1: What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved digital health products for their patients?

Pfizer Comment: Provider adoption of DMHTs and PDTs is limited due to uncertainty around reimbursement, workflow integration, and evidence expectations. CMS can support provider engagement by:

- Ensuring adequate and reliable reimbursement for DMHTs that reflects clinical value and development cost.
- Developing provider toolkits that highlight DMHTs as practice-extending solutions, especially for behavioral health and chronic condition management in underserved areas.
- Including DMHTs in eligible services for APMs and MIPS improvement activities, aligning provider incentives with digital adoption.

Question TD-1: "What short-term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?"

Pfizer Comment: Stimulating developer interest requires reimbursement predictability, coding transparency, and clear market pathways. CMS can support this by:

- Over the short term, setting adequate national price floor for DMHT codes, specifically G0552, to ensure adequate reimbursement across MAC jurisdictions. Over the long term, establishing product-specific pricing for PDTs.
- Expanding the scope of eligible PDTs to include new clinical domains such as gastroenterology, cardiology, and metabolic diseases, where digital therapeutics are emerging. CMS should acknowledge and support coverage for PDTs authorized as multiple FDA classifications beyond '*Computerized behavioral therapy device for psychiatric disorders*'[4].
- Partnering with FDA to clarify evidentiary expectations for mental health tools by considering whether evidence thresholds appropriate for DMHT that demonstrate real world impact on patient engagement, symptom control, and functional outcomes, align with traditional pharmacologic endpoints.

---

[1] https://phti.org/assessment/virtual-solutions-anxiety-depression/
[2] https://www.novitas-solutions.com/webcenter/portal/MedicareJH/FeeLookup
[3] https://medicare.fcso.com/SharedTools/faces/FeeSchedule_en.jspx?lob=Part%20B&state=FL
[4] FDA Product Classification 882.5801

- Encouraging Medicare Advantage plans to cover PDTs beyond DMHTs as a supplemental benefit.

Thank you for considering these comments and for your efforts to ensure that all patients have access to the care they need. If you have questions or need additional information, please contact Abram Bieliauskas at Abram.Bieliauskas@pfizer.com.

Sincerely,

Margaret Davis-Cerone
Senior Director, Head of Federal Policy
Corporate Affairs
US Policy & Government Relations